

Patient Cohorting Tool

User Manual

Barts Life Sciences

Version 1.0

RESEARCH COHORT DISCOVERY / USER GUIDE

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At a glance

Purpose

The Patient Cohorting Tool supports early research feasibility assessment by demonstrating the number and demographic breakdown of patients that meet a proposed study's criteria. It should be used before making a Data Access Request (DAR); it does not replace the DAR process or provide access to patient-level records.

Data source and refresh

The tool is hosted on the Barts Health Data Platform. Its clinical data originate from Oracle Millennium and are refreshed monthly; results are not real-time.

Build a cohort

Enter a cohort title and choose current-age, gender and ethnicity criteria, an optional admission date range, and parent SNOMED CT findings/disorders to include or exclude. Diagnosis-specific dates can be set independently of admission dates.

Read results carefully

SNOMED CT searches include ICD-10 diagnoses which have been mapped to SNOMED CT; clinical coding and mappings may be incomplete. Counts support feasibility planning rather than definitive case ascertainment.

Access and privacy

Production requests require an eligible NHS or QMUL email address; results are sent by email. Small counts are suppressed (fewer than 10 shown as 0), and counts of 10 or more are rounded to the nearest 10.

Support

bartshealth.bls.cohortingtool@nhs.net

1. Introduction

The Patient Cohorting Tool is a research feasibility tool developed by the Data Science team at Barts Life Sciences to support the identification, and demonstrate the diversity, of potential patient cohorts for research studies using routinely collected clinical data.

The tool allows researchers to define a cohort using demographic, admission and clinical criteria through a web-based interface, without needing to write SQL or directly query the underlying clinical database. Returned results are anonymous aggregate values, which allows the tool to be used before research governance approvals are in place.

Users can define criteria including:

- Age
- Gender
- Ethnicity
- Admission date range
- Diagnoses or clinical findings that patients **must have**
- Diagnoses or clinical findings that patients **must not have**
- Optional timeframes associated with individual diagnoses or findings

Clinical findings and disorders are selected using **SNOMED CT terminology**. The tool automatically includes SNOMED concept descendants when a clinical concept is selected, helping users to easily create clinically meaningful cohort definitions.

The current version of the tool is intended to support **research cohort discovery and feasibility assessment**.

2. Access to the Tool

The production version of the Patient Cohorting Tool can be used to submit cohort requests using an eligible:

- NHS email address (@nhs.net)
- Queen Mary University of London email address (@qmul.ac.uk)

Requests submitted using other email domains are not accepted directly by the application. Researchers who do not have an eligible email address can contact the Barts Life Sciences Data Science team at bartshealth.bls.cohortingtool@nhs.net to discuss their requirements.

3. Demo and Production Versions

The tool can operate in two modes.

3.1 Demo version

The demonstration environment is intended to illustrate how the Cohort Builder works. The data and results presented in the demo environment are illustrative and are **not real clinical data**. When a demo request is submitted, the results are displayed directly in the application.

3.2 Production version

The production version runs cohort queries against the Barts Health Data Platform research curated datasets. After a valid request is submitted, the application confirms that the submission has been received. The request is then processed, and the results are sent to the email address supplied by the user.

4. Creating a Cohort

The cohort form contains several sections that can be used together to define the population of interest. Not every criterion is required.

A cohort may therefore be broad or highly specific depending on the research question.

5. Cohort Title

A **Cohort Title** is required for every request.

The title:

- must contain at least 5 characters
- may contain letters
- may contain numbers
- may contain spaces
- may contain hyphens (-)
- may contain underscores (_)

Other special characters are not accepted.

A meaningful title is recommended because it helps identify the request and associated results.

Example

Diabetic Foot Feasibility 2025

6. Demographic Criteria

6.1 Gender

Users can select one or more gender categories to refine their cohort. Gender selection is optional. If no category is selected, the search includes patients of all recorded genders, including those whose gender is not stated or is unknown.

The gender categories are based on the [NHS Data Dictionary's Gender Identity Code classification](#), which includes:

- Male (including trans man)
- Female (including trans woman)
- Non-binary
- Other (not listed)
- Not stated
- Not known (not recorded)

6.2 Age

A minimum and maximum age can be specified. By default, **the minimum age is set to 18 years and the maximum age to 120 years** when the tool is opened. Users can adjust these values to specify their desired age range.

The age filter uses the patient's current age at the time the cohort query is run, not their age at admission or at diagnosis. An admission date range filters admission dates independently and it does not recalculate age for each admission. For example, a patient currently aged 41 may be included in a search for ages 40–75 and admissions during 2020, even if they were younger than 40 when admitted. Conversely, a patient whose current age is outside the selected range is excluded even if their age at an eligible historical admission was within it.

6.3 Ethnicity

Ethnicity selection is optional. Users can select one or more ethnicity categories to refine their cohort. If no category is selected, the search includes patients from **all recorded ethnic categories**, including those whose ethnicity is not stated or is unknown.

The ethnicity categories are based on the [NHS Data Dictionary's Ethnic Code classification](#), which includes:

- White
- Mixed
- Asian or Asian British
- Black or Black British
- Chinese or Other Ethnic Group
- Not Stated

Each category includes more specific ethnic classifications, allowing users to refine their cohort according to the available ethnicity data.

7. Admission Time Range

An optional admission date range can be specified using **From** and **To**. These dates relate to the patient's admission date.

Example**From:** 01 January 2025**To:** 31 December 2025

This restricts the cohort to qualifying admissions occurring during 2025. Either date can also be left blank.

For example:

From: 01 January 2024**To:** Any

means that admissions occurring from 1 January 2024 onwards may be considered.

The application prevents the start date from being later than the end date.

Age and admission dates are separate filters: admission dates are evaluated against the selected date range, while age is evaluated as current age. The tool does not currently offer an age-at-admission or age-at-diagnosis filter.

8. SNOMED CT Codes

Clinical findings and disorders within the Patient Cohorting Tool are defined using **SNOMED CT**. The underlying clinical data originate from clinician input problems lists input into Oracle Millennium. Clinical coder produced ICD-10 diagnosis codes are also used in the source data and are mapped to SNOMED CT for searching in this tool. The presence of a mapped SNOMED CT concept does not necessarily mean the diagnosis was originally entered in Millennium by a clinician.

Coding quality and mapping coverage vary by condition, clinical workflow and time period. Some diagnoses may be missing, coded differently or mapped to a broader or less-specific concept. Therefore, the tool is suitable for initial research feasibility assessment but a SNOMED-based cohort count should not be treated as a validated count of all patients with a condition. For studies requiring high diagnostic accuracy, the proposed code set and mapping should be reviewed with the clinical/data team and confirmed during the appropriate data-access process.

Users do not need to know the SNOMED CT code in advance. Instead, they can search using a clinical term and select the most appropriate concept from the search results.

For example, entering “diabetic”, the drop-down menu may return different SNOMED CT concepts among which there is “**Diabetic foot**”. When the user selects the concept, the button “Add” should be clicked to add this concept to the selected criteria. Once added, the concept will appear as a clickable link (please see section 8.5 Checking a SNOMED Concept).

When a concept is added, the tool can retrieve the selected concept together with its associated descendant concepts. This allows a broader clinical concept to represent

several more specific diagnoses without requiring the user to search for every code individually.

Each SNOMED code included in the cohort definition is classified by the tool as one of the following:

- **Primary code**
- **Child code**
- **Not active**

These classifications are described below.

8.1 Primary Code

The **Primary code** is the SNOMED CT concept directly selected by the user.

For example, if the user searches for “diabetic” and selects: **Diabetic foot** then that selected concept is classified as **Primary code**

The primary code represents the clinical concept the user explicitly chose when defining the cohort.

8.2 Child Codes

A **Child code** is a more specific SNOMED CT concept that falls under the selected primary concept within the SNOMED hierarchy. Child codes and all descendant codes are automatically retrieved by the tool when applicable. This means that users do not need to identify every more specific diagnosis individually.

For example, a selected diabetic-foot concept may include more specific related concepts such as:

SNOMED concept	Code type
Diabetic foot	Primary code
Skin ulcer of toe due to diabetes mellitus type 1	Child code
Skin ulcer of toe due to diabetes mellitus type 2	Child code

The inclusion of child codes is particularly useful when the underlying clinical data may have been coded using a more specific concept than the broader term selected by the researcher.

8.3 Not Active Codes

SNOMED CT terminology changes over time. Some concepts that were used historically are subsequently made inactive and replaced, retired or otherwise removed from active use in newer SNOMED releases. These concepts may still appear in historical clinical records.

For this reason, the Patient Cohorting Tool **retains inactive SNOMED codes rather than automatically removing them.**

Inactive concepts are classified as “**Not active**”

Example

SNOMED concept	Code type
Diabetic foot	Primary code
Foot abnormality – diabetes-related	Not active
Skin ulcer of toe due to diabetes mellitus type 1	Child code

A concept labelled **Not active** is therefore still included in the cohort search. The label means that the SNOMED concept is no longer active in the terminology version being queried. The inclusion of inactive codes is particularly important when searching longitudinal clinical datasets, where historical records may contain terminology that is no longer actively used. A Not active code can still be used to identify relevant historical records.

8.4 Number of Included SNOMED Codes

When a concept is selected, the interface displays the number of SNOMED codes associated with that criterion. For example, “**Diabetic foot (Include 7 codes)**” means that seven SNOMED codes are being considered for that clinical criterion. These may include a combination of the primary code, child codes and inactive historical codes.

This type of information should give users an indication of how broad the selected SNOMED concept is.

8.5 Checking a SNOMED Concept

After a finding has been added, the selected SNOMED concept appear as a clickable link that will redirect the SNOMED CT browser page for that specific concept. Users can follow this link to inspect the concept if they want to confirm that the selected terminology represents the intended clinical condition and it is appropriate for their intended cohort definition. The link should also provide additional information, including its clinical definition of the concept, associated terminology, and position within the SNOMED CT hierarchy.

9. Clinical Inclusion and Exclusion Finding/Disorder

The tool provides two sections for defining clinical criteria:

- **Must HAVE Finding / Disorder**
- **Must NOT HAVE Finding / Disorder**

Both use SNOMED CT terminology to identify findings or disorders (please see section 8. SNOMED CT Codes).

To add a finding/disorder:

1. Start typing the required clinical term.
2. Review the SNOMED search results.
3. Select the appropriate SNOMED concept.
4. Optionally specify a **From** and/or **To** date.
5. Click **Add**.
6. Review the selected criterion displayed underneath the search box.

The selected item shows:

- the selected clinical concept
- the number of SNOMED codes included
- the selected timeframe

A selected finding or disorder can be removed before submission by clicking the  button beside it.

Removing the finding removes its corresponding SNOMED codes from the cohort definition.

9.1 Must HAVE Finding / Disorder

Use **Must HAVE** to define diagnoses or clinical findings that a patient must have to qualify for the cohort. For example, a researcher wants to identify patients with diabetic foot, they should select “**Must HAVE: Diabetic foot**”. The tool will then identify the corresponding population using the selected primary SNOMED code together with the associated child and inactive codes included in that concept expansion.

Additional clinical findings or disorders can be added by repeating the same process. When multiple concepts are selected, the tool will identify patients who meet all the specified inclusion criteria.

9.2 Must NOT HAVE Finding / Disorder

Use **Must NOT HAVE** to define diagnoses or findings that should exclude a patient from the cohort. For example, if a researcher wants patients with diabetic foot but wants to exclude patients with a particular condition (i.e., pregnancy), they should specify “**Must HAVE: Diabetic foot**” and “**Must NOT HAVE: pregnancy**”. The tool will then identify the corresponding population and will exclude patients matching the exclusion criterion.

Additional exclusion criteria can be added by repeating the same process. When multiple concepts are selected, patients matching any of the specified exclusion criteria will be excluded from the resulting cohort.

10. Diagnosis-Specific Timeframes

Each selected SNOMED finding can optionally have its own timeframe. After selecting a SNOMED term, use “**From**” and “**To**” before clicking “**Add**” and add the SNOMED concept to the search. This timeframe relates to the diagnosis or finding and is separate from the overall Admission Time Range.

Example

A study wants to identify patients admitted during 2025 who have had a diabetes diagnosis recorded since 2020.

The user could specify:

Admission Time Range

- **From:** 01 January 2025
- **To:** 31 December 2025

Must HAVE Finding / Disorder

- **Diabetes**
- **From:** 01 January 2020
- **To:** Any

This means that the admission must occur during 2025, while the diabetes diagnosis can have been recorded at any point from 2020 onwards.

11. Combining Criteria

Different criteria can be combined to create a specific cohort.

For example:

- Age: 40–75
- Gender: Female
- Admission period: 2024–2026
- Must HAVE: Diabetic foot, Diabetes
- Must NOT HAVE: Pregnancy

The resulting cohort contains patients satisfying the required demographic, admission and clinical conditions while excluding patients matching the defined exclusion criteria.

12. Summary of Selected Criteria

Before submitting a request, the user should review the **Summary of Selected Criteria** shown underneath the form.

The summary includes:

- cohort title
- email address, where applicable

- gender
- age range
- ethnicity
- admission timeframe
- Must HAVE findings/disorders and diagnosis-specific timeframes
- Must NOT HAVE findings/disorders and diagnosis-specific timeframes

This provides an opportunity to verify the cohort definition before submission.

13. Submitting a Request

Once the cohort definition has been reviewed, the user should click “**Submit**”. The tool validates the required information before accepting the request.

A confirmation window will then appear, asking the user to confirm that they have reviewed the selected criteria in the summary. The user can choose between two options:

- No, go back and review: Closes the confirmation window and allows the user to review or modify the selected criteria before submission.
- Yes, submit request: Confirms the selected criteria and submits the cohort request.

In the production version of the tool, a successful submission displays: “**Submission received**. The page can then be closed”. The cohort request will be processed and the results sent to the eligible email address supplied by the user (please see section 19. Receiving Production Results).

In the demo version of the tool, a successful submission displays will redirect the user to the result page.

14. Understanding the Results

Depending on the selected cohort and environment, the output can contain information including:

- total number of individual patients
- total number of encounters
- Table and bar graph of gender distribution (unique patient count)
- Table and bar graph of age distribution (unique patient count) Table and pie chart of ethnicity distribution (unique patient count)
- Table of admissions by month-year
- Number of encounters per findings/ disorder included, code type and specific timeframe
- List of findings/ disorder excluded

The results may also include visual summaries of demographic information.

15. Disclosure Control

Disclosure-control rules are applied to reduce the risk of identifying patients from small groups:

- counts below **10** are displayed as **0**
- counts of 10 or greater are rounded to the nearest: **10**

Examples

Actual count	Displayed count
4	0
9	0
12	10
27	30
44	40

A displayed value of **0** **therefore does not necessarily mean that no patients exist**. It may mean that the true value is below the disclosure threshold.

If the overall cohort contains fewer than 10 patients, the displayed patient count is set to zero and detailed demographic breakdowns are suppressed. This helps protect against disclosure of small patient groups.

16. Receiving Production Results

Production results are sent to the email address entered when the cohort request was submitted. The email contains an HTML results file.

To review the report:

1. Open or save the attached HTML file.
 - Open it using a standard web browser, such as Microsoft Edge, Google Chrome or Mozilla Firefox
2. Review the cohort results.

17. Query Processing Time

Processing time depends on the complexity of the cohort definition and the amount of clinical data that needs to be searched.

Queries may take longer where they involve:

- very broad patient populations

- common diagnoses
- long date ranges
- large numbers of SNOMED descendant codes
- several inclusion or exclusion criteria

Production requests are processed separately from the interactive webpage, so the user does not need to keep the page open once the request has been successfully submitted.

18. Failed request

If a cohort request fails during processing, an email notification will be sent to both the user and the Barts Life Sciences Data Science team.

Before submitting another request, users should review their selected criteria and consult this user manual to ensure that the cohort definition is correct.

If the problem persists or the cause is unclear, users can contact the Barts Life Sciences Data Science team at bartshealth.bls.cohortingtool@nhs.net, providing the cohort title and a brief description of the issue.

19. Troubleshooting

19.1 SNOMED terms are not loading

If a SNOMED concept does not appear in the drop-down menu after the concept is typed:

1. Refresh the page.
2. Try the search again.
3. Try using a shorter or more general search term.
4. Try again after approximately five minutes.
5. Contact the Barts Life Sciences Data Science team if the problem persists.

19.2 The requested SNOMED concept cannot be found

Try using:

- a shorter clinical phrase
- a synonym
- a broader clinical concept

The SNOMED terminology browser link can also be used to inspect available concepts.

19.3 A result is shown as zero

A result of zero may mean:

- there were no qualifying patients, or
- the number of qualifying patients was below 10 and has therefore been suppressed for disclosure-control purposes.

20. Recommended Approach to Building a Cohort

For more complex research questions, it may be easier to construct the cohort progressively.

○ **Step 1 – Define the population**

Set:

- age
- gender, if required
- ethnicity, if required
- admission timeframe

○ **Step 2 – Add the principal clinical criterion**

For example:

Must HAVE: Diabetic foot

○ **Step 3 – Review the SNOMED concept**

Confirm that the selected concept accurately represents the intended condition.

○ **Step 4 – Add additional inclusion criteria**

Add further findings or disorders if necessary.

○ **Step 5 – Add exclusions**

Use **Must NOT HAVE** for relevant clinical exclusion criteria.

○ **Step 6 – Add diagnosis-specific dates**

Use diagnosis-specific timeframes where the timing of a finding is important.

○ **Step 7 – Review the summary**

Confirm that the selected criteria represent the intended research population.

○ **Step 8 – Submit**

Submit the request and retain the cohort title for reference.

21. Example Cohort

Research question: Identify adults aged 40–80 admitted during 2025 who have a diabetic-foot diagnosis.

Patient Cohort Tool Criteria:

- **Cohort Title:** Diabetic Foot Feasibility 2025
- **Age:** 40–80

- **Gender:** All
- **Ethnicity:** All
- **Admission Time Range:** 01 January 2025 – 31 December 2025
- **Must HAVE:** Diabetic foot
- **Diagnosis Timeframe:** Any
- **Must NOT HAVE:** None

22. Interpreting Results

Several factors should be considered when interpreting cohort counts.

Clinical coding

The tool identifies records based on the information available in the underlying clinical datasets. The absence of a coded diagnosis does not necessarily establish that the patient has never had the condition.

SNOMED hierarchy

Selecting a parent concept can include multiple more specific child concepts. Users should therefore check that the selected primary concept represents the intended clinical condition.

Historical terminology

Inactive SNOMED concepts can still occur in historical clinical records and are retained where relevant.

Disclosure control

Counts below ten are displayed as zero and other counts are rounded to the nearest ten. The results should therefore be interpreted primarily as **research feasibility estimates rather than exact patient counts**.

23. Data and Governance

The Patient Cohorting Tool is designed to support research cohort discovery and feasibility. The Tool is part of the Barts Health Data Platform. Data originate from Oracle Millennium and are refreshed on a monthly basis. Researchers can use it to assess whether a suitable cohort may exist before submitting a Data Access Request (DAR). A DAR and any relevant approvals are still required to obtain access to underlying data.

Users should only submit searches for appropriate research purposes and should interpret the outputs within applicable research, information-governance and data-access arrangements.

The tool does not provide users with direct access to the underlying clinical database. Instead, the researcher defines a cohort through the interface and the application constructs and processes the corresponding query.

24. Quick Reference

Field	Required?	Behaviour
Cohort Title	Yes	Minimum 5 characters
Email	Production	NHS or QMUL address
Gender	No	All categories if none selected
Minimum Age	Yes	Minimum cohort age (current age)
Maximum Age	Yes	Maximum cohort age (current age)
Ethnicity	No	All categories if none selected
Admission From	No	Earliest qualifying admission
Admission To	No	Latest qualifying admission
Must HAVE	No	Clinical inclusion criterion
Must NOT HAVE	No	Clinical exclusion criterion
SNOMED From	No	Earliest diagnosis date for that finding
SNOMED To	No	Latest diagnosis date for that finding

25. SNOMED Code Types – Quick Reference

Code type	Meaning
Primary code	SNOMED concept directly selected by the user
Child code	Descendant SNOMED concept automatically included by the tool
Not active	Historical or inactive SNOMED concept retained so that relevant historical clinical records can still be identified

26. Frequently Asked Questions

Is SNOMED coding in Oracle Millennium reliable enough to depend on?

The source data include clinical coding from **Oracle** Millennium, including ICD-10 diagnoses mapped into SNOMED CT using terminology mappings provided through the [SNOMED CT UK Data Migration Workbench](#) from the NHS TRUD. Coding completeness, clinical specificity

and mapping accuracy cannot be assumed for every condition. The resulting counts should be used as feasibility estimates.

How does the age filter interact with an admission date range?

The two filters are applied independently. The admission range selects qualifying admissions by admission date, while the age filter uses each patient's current age when the query is run. A patient may have been younger or older than the selected age range at the time of a historical admission.

What age is reported in the results?

Current age at the time the cohort query is run and not age at admission and not age at diagnosis. The same interpretation applies to the age filter and reported age summaries.

Is the data real-time, and where does the tool sit in the data-access process?

No. The tool is part of the Barts Health Data Platform. Data originate from Oracle Millennium and are refreshed monthly. Use the tool for research feasibility assessment before making a Data Access Request (DAR). A DAR and relevant approvals remain necessary for access to underlying data.

Do I need to know SNOMED codes?

No. Search using clinical terminology and select the appropriate concept from the results.

What is a Primary code?

It is the SNOMED concept that you directly selected.

What is a Child code?

It is a more specific concept beneath the selected primary concept that has been automatically included by the tool.

What does Not active mean?

The SNOMED concept is no longer active in the current terminology release but may still exist in historical clinical records.

Does Not active mean the code is ignored?

No. Not active codes are retained and can still be searched.

Do I need to add every child SNOMED concept manually?

No. Relevant descendant concepts are automatically included.

What happens if I select no gender?

All available gender categories are considered.

What happens if I select no ethnicity?

All available ethnicity categories are considered.

Can I search without specifying an admission date?

Yes.

Can each diagnosis have its own date range?

Yes. Individual findings can have their own From and To dates.

Why does a result show zero patients?

There may genuinely be no qualifying patients, or the actual patient count may be below the disclosure threshold of ten.

Are the demo results real patient data?

No. Demo results are illustrative only.

Where are production results sent?

They are sent to the eligible email address supplied when the cohort request is submitted.

27. Support

For technical problems, questions about using the tool, feedback, or requests from users who cannot submit directly through the application, contact:

Barts Life Sciences Data Science Team**Email:**

bartshealth.bls.cohortingtool@nhs.net

When reporting an issue, where possible include:

- the cohort title
- a brief description of the problem
- the stage at which the issue occurred
- any error message displayed
- a screenshot, where appropriate

Do not include patient-identifiable information in a support request unless specifically instructed through an approved governance process.

Document Control

Document: Patient Cohorting Tool User Manual

Owner: Barts Life Sciences

Version: 1.0

Purpose: User guidance for research cohort discovery and feasibility

Support: bartshealth.bls.cohortingtool@nhs.net